



Public Health Emergency Operations Center

"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°081/MVE B_03/08/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (51)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (12/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Musienene and Lubero. Masereka • , - South Kivu (1/34): Miti-Murhesa • Tshopo (5/23): Kabondo, Lubunga, Makiso-Kisangani, Mangobo and Wanie-Rukula
Reporting date	August 3, 2026
Publication date	August 4, 2026

KEY INDICATORS — CUMULATED AS OF 03 AUGUST 2026

CONFIRMED CASES — 5 PROVINCES 3,874 +72 new cases (Ituri 58 · N-Kivu 12 · Haut-Uélé 2)	CONFIRMED DEATHS · LEATHITIS 1,751 · (45.1%) +44 deaths today (Ituri 35 · N-Kivu 5 · H-Uélé 4)	SUSPECTED CASES OF THE DAY 301 Ituri 182 · North Kivu 109 · Upper-Uélé 6 · Tshopo 2 · South Kivu 2
PATIENTS IN ISOLATION 717 Ituri 471 · N-Kivu 196 · Haut-Uélé 31 · Tshopo 3 · South Kivu 16	CURED — CUMULATIVE 749 +22 recovered (Ituri 21 · Haut-Uélé 1)	NATIONAL CONTACT TRACKING 78.4% 14,300 / 18,243 views · Ituri 80.2% · N-Kivu 76.0% · H-Uélé 63.0% · Tshopo 81.7% · South Kivu 100%

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones have been affected in the last 24 hours.

The epidemic remains confined to 51 out of 140 health zones, spread across five provinces: Ituri (28/36), North Kivu (12/34), Haut-Uélé (5/13), Tshopo (5/23), and South Kivu (1/34). North Kivu has 73 affected health zones out of 684, and Haut-Uélé has 22 out of 213.

- **Case trends:** In the last 24 hours, 72 new confirmed cases, 44 confirmed deaths, and 22 recoveries have been recorded, bringing the totals to 3,874 confirmed cases, 1,751 deaths, and 749 recoveries, representing a case fatality rate of 45.1%. Today's deaths include 30 community deaths confirmed by post-mortem testing (66.7%) and 14 deaths occurring in treatment centers.

In Ituri, 58 new confirmed cases were reported by the laboratory, identified in Rwampara (19), Bunia (11), Nia-Nia (8), Nizi (8), Bambu (5), Tchomia (2), Komanda (1), Lita (1), Logo (1), Mangala (1), and Mongbwalu (1), along with 35 deaths, 25 of which were community deaths (71.4%). Six repeat positive tests were also reported, bringing the total number of positive cases for the day to 64.

- o **North Kivu:** 12 new confirmed cases in Katwa (9), Butembo (2) and Musienene (1), and 5 deaths, all recorded in Katwa, including 2 in Ebola Treatment Centers (ETCs). The cumulative total reaches 426 cases and 288 deaths, representing a case fatality rate of 68.1%.
- o **Haut-Uélé :** 2 new confirmed cases in Isiro, both deceased and confirmed by Swab, as well as 2 in-hospital deaths reported in Boma Mangbetu and Pawa. The province has a total of 63 confirmed cases and 38 deaths, representing a case fatality rate of 60.3%, spread across 5 out of 13 health zones and 22 out of 213 health areas. The confirmed case who escaped from Gbonzunzu has been found in the Adjangwe health area.
- o **Tshopo :** No new confirmed cases or deaths have been reported. The provincial total remains at 7 confirmed cases.
5 deaths and 1 recovery, representing a case fatality rate of 71.4%.
- o **South Kivu :** The provincial total remains at 3 confirmed cases, including 1 death and 2 recoveries. The province has gone 54 days without a new confirmed case since June 10, 2026. Contact tracing continues: 78 contacts listed, 23 discharged, and 55 still being monitored; all have been seen (100%).
- **Analysis:** Ituri remains the epicenter with 87.1% of cumulative cases and 80.6% of today's confirmations, but its case fatality rate (42.0%) remains similar to that of North Kivu (67.6%), which accounts for 16.7% of new cases. This discrepancy reflects very late detection in North Kivu, where 51 of the 109 suspected cases today resulted in deaths (46.8%) and where the mortality rate within treatment centers reached 35%. Nationally, 28 of the 44 deaths today (63.6%) occurred in the community: transmission is still largely escaping the early detection system. Contact tracing is at 78.4%, compared to 79.2% the previous day, below the operational threshold of 95%. This decline is very marked in the province of Ituri (80.2%), where Bunia (32.2%) and Mangala (53.0%) alone account for 1,771 of the 2,289 contacts not seen.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 51 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.



Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	3,375	1,417	42.0%	28 / 36 (77.8%)	58
North Kivu	426	290	68.1%	12 / 34 (35.3%)	12
Haut-Uélé	63	38	60.3%	5/13 (38.5%)	2
Tshopo	7	5	71.4%	5/23 (21.7%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,874	1,751	45.1%	51 out of 140	72

Case fatality rate recalculated from daily totals. No new health zones have been affected in the last 24 hours.

Ituri province accounts for 87.1% of confirmed cases (n = 3,375) and 81.0% of deaths (n = 1,417), with a case fatality rate of 42.0%. The health zones of Bunia (925 cases), Rwampara (674), Mongbwalu (553), Nizi (410), Lita (142), and Nyankunde (115) represent 83.5% of the cases reported at the provincial level and 72.8% of the national total. North Kivu, the second-largest outbreak hotspot, saw a 2.9% increase in cases in a single day and maintains a higher case fatality rate than Ituri.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3,375	1,417	42.0%	58	25	10	35
Adja	11	1	9.1%	0	0	0	0
Aru	5	5	100.0%	0	0	0	0
Ariwara	7	2	28.6%	0	0	0	0
Aungba	10	5	50.0%	0	0	0	0
Bamboo	69	15	21.7%	5	0	0	0
Boga	1	1	100.0%	0	0	0	0
Bunia	925	277	29.9%	11	5	2	7
Damascus	18	8	44.4%	0	0	0	0
Drodro	11	6	54.5%	0	0	0	0
Fataki	35	17	48.6%	0	0	0	0
Gethy	1	0	0.0%	0	0	0	0
Kambala	2	0	0.0%	0	0	0	0
Kilo	29	11	37.9%	0	0	0	0
Komanda	38	27	71.1%	1	0	0	0
Lita	142	86	60.6%	1	1	1	2
Logo	10	5	50.0%	1	0	1	1
Lolwa	7	2	28.6%	0	0	0	0
Mahagi	4	1	25.0%	0	0	0	0
Mambasa	10	6	60.0%	0	0	0	0
Mandima	16	12	75.0%	0	0	0	0

Mangala	106	67	63.2%	1	1	0	1
Mongbwalu	553	264	47.7%	1	0	1	1
Nia-Nia	112	63	56.3%	8	5	1	6
Nizi	410	201	49.0%	8	6	1	7
Nyankunde	115	34	29.6%	0	0	0	0
Rimba	9	4	44.4%	0	0	0	0
Rwampara	674	273	40.5%	19	7	3	10
Tchomia	45	24	53.3%	2	0	0	0
North Kivu	426	290	68.1%	12	3	2	5
Blessed	64	49	76.6%	0	0	0	0
Butembo	89	69	77.5%	2	0	0	0
Goma	1	0	0.0%	0	0	0	0
Kalunguta	7	4	57.1%	0	0	0	0
Katwa	195	125	64.1%	9	1	2	3
Kyondo	13	10	76.9%	0	0	0	0
Lubero	1	1	100.0%	0	0	0	0
Mabalako	1	0	0.0%	0	0	0	0
Masereka	5	1	20.0%	0	0	0	0
Musienene	44	25	56.8%	1	0	0	0
Oicha	4	3	75.0%	0	0	0	0
Vuhovi	2	1	50.0%	0	0	0	0
Haut-Uélé	63	38	60.3%	2	2	2	4
Boma Mangbetu	13	5	38.5%	0	0	1	1
Isiro	21	13	61.9%	2	2	0	2
Pawa	12	11	91.7%	0	0	1	1
Rungu	1	0	0.0%	0	0	0	0
Wamba	16	9	56.3%	0	0	0	0
Tshopo	7	5	71.4%	0	0	0	0
Kabondo	1	1	100.0%	0	0	0	0
Lubunga	1	0	0.0%	0	0	0	0
Makiso-Kisangani	3	3	100.0%	0	0	0	0
Mangobo	1	1	100.0%	0	0	0	0
Wanie-Rukula	1	0	0.0%	0	0	0	0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%	0	0	0	0
Total	3,874	1,751	45.1%	72	30	14	44

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Lita and Nyankunde, with transmission continuing in the areas already affected without any new geographical expansion in the last 24 hours.

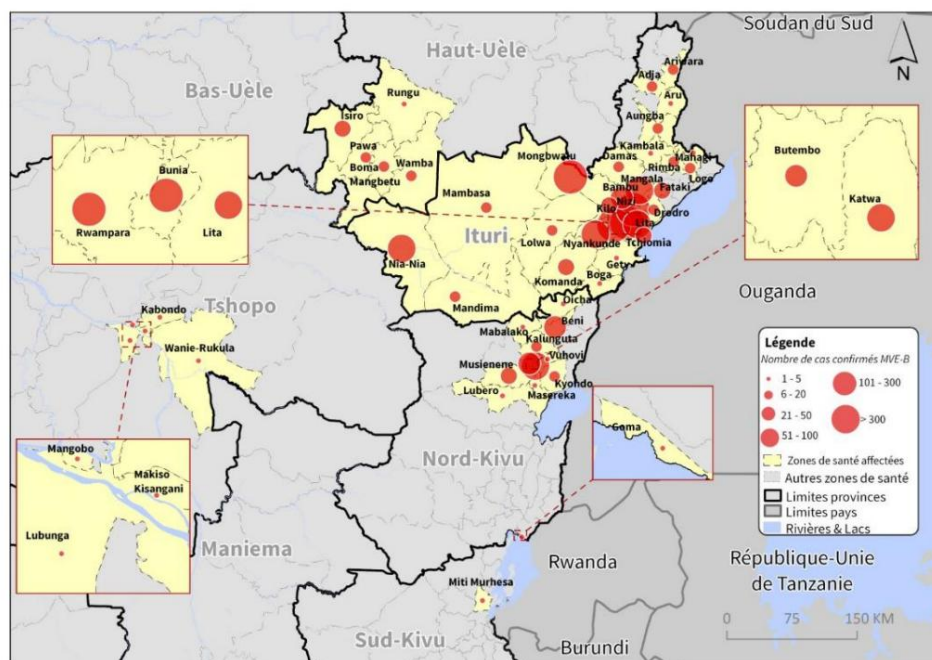


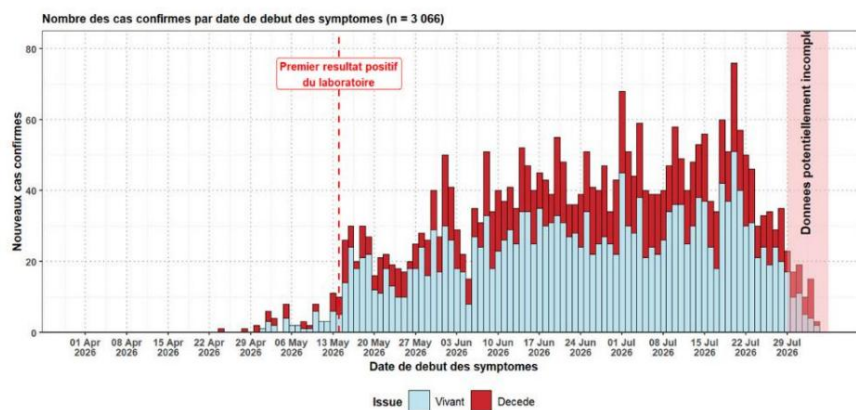
FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission in already affected health zones underscores the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to notification delays and the gradual consolidation of data.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION



Source : DHIS2 Tracker - Riposte MVE, août 2026. Données incluses : cas confirmés avec statut vital et date de début de symptômes renseignés, n = 3 066.

FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

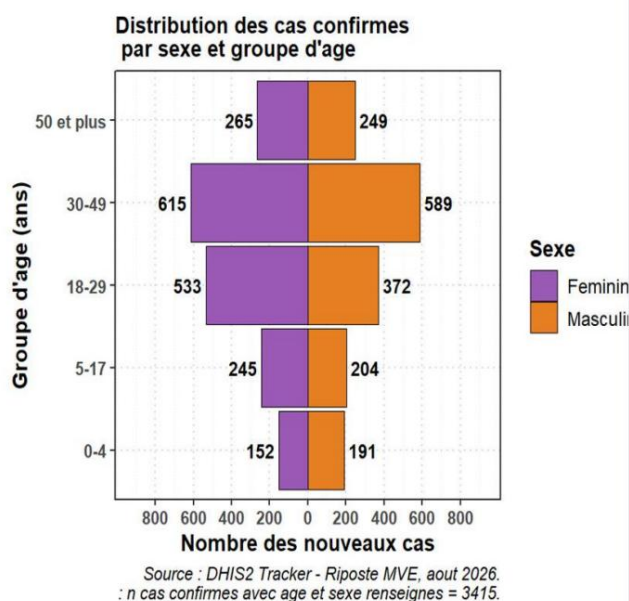
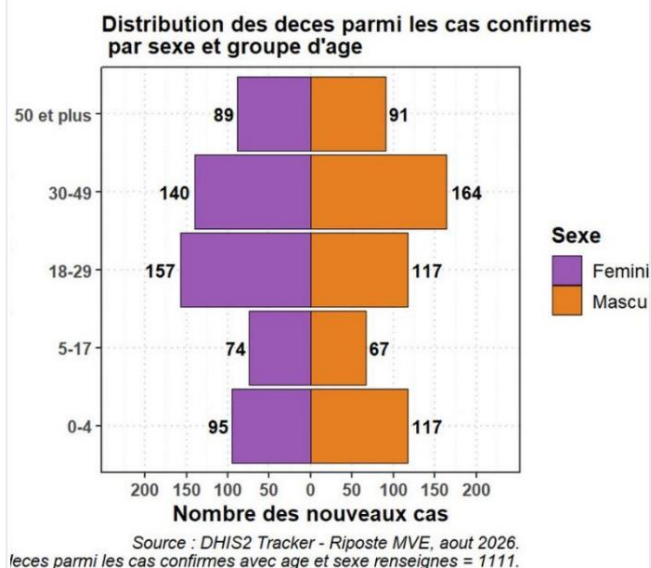


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	16	0	1	0	2	19
New alerts received	540	703	5	15	8	1271
Total alerts for the day	556	703	6	15	10	1290
Alerts investigated	344	687	6	15	10	1062
Investigation rate (24 h)	61.9%	97.7%	100.0%	100.0%	100.0%	82.3%
Alerts confirmed — live	126	57	0	2	2	187
Alerts confirmed — deceased (comm.)	56	40	6	0	0	102
Total suspected cases for today	182	97	6	2	2	289

In total, 1,290 alerts were managed (including 1,271 new ones) and 1,260 investigated within 24 hours (82.3%), resulting in 289 confirmed suspected cases (187 living and 102 community deaths). North Kivu accounts for the majority of these with 703 alerts, of which 344 were investigated (97.7%): provincial performance has declined significantly, with Lita validating only 5.6% of its 36 alerts, while Rwampara reached 87.8% of its 77. Haut-Uélé investigated its 6 alerts, Tshopo its 15, and South Kivu its 10, all at 100%.

3.2 Laboratory: tests and positivity

ŷ **Ituri** : Ituri: 275 samples received and tested, of which 64 were positive and 6 were re-sampled, representing a positivity rate of 23.3%.

ŷ **North Kivu**: 106 samples received, of which 46 were analyzed in Beni (none positive) and 60 in Butembo (14 positive: 12 new cases) (including 3 deaths and 2 re-samples), representing a positivity rate of 13.2%; 1 analysis remains pending. Receipt of 10 Altona RT-PCR kits (ASLM), Biofire PCR kits, and a Biofire Torch machine (US CDC).

ŷ **Haut-Uélé**: Haut-Uélé: 2 positive post-mortem samples, both in Isiro, no invalid results. The machine intended for the Wamba diagnostic laboratory is available and its installation is scheduled.

ŷ **Tshopo**: Tshopo: no results were returned for the day; 12 samples of suspected cases were received and unpacked. originating from Bas-Uélé. The provincial cumulative total remains at 7 positive cases.

ŷ **South Kivu**: South Kivu: 5 samples received (Nyangezi 4, Bagira 1), all negative. The provincial total stands at 3 positive results out of 298 samples analyzed (1.0%); 94% of samples are analyzed within one day of collection and 5 exams are still pending.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	11,544	9,255	80.2%
North Kivu*	6021	4,576	76.0%
Haut-Uélé**	508	320	63.0%
South Kivu	55	55	100.0%
Tshopo***	115	94	81.7%
Total	18,243	14,300	78.4%

Contact tracing has declined to 78.4% nationally, compared to 79.2% the previous day, and remains well below the operational threshold of 95%. Ituri is monitoring 9,255 contacts out of 11,544 (80.2%) and continues to see an increase in volume—665 new contacts listed —, but two areas account for 77.4% of the 2,289 unseen contacts: Bunia (998 but not yet seen (32.2% follow-up). Mangala has 773 contacts not yet seen (53.0%), followed by Nizi (114) and Tchomia (57). The Damas, Fataki, and Kambala zones remain at zero listed contacts despite having cumulative cases. North Kivu has 4,576 contacts seen out of 6,021 (76.0%), with 155 lost to follow-up and 720 never seen; Katwa (67%), Beni (70%), and Butembo (77%) are dragging down the provincial performance. Haut-Uélé is only following up with 320 of its 508 contacts (63.0%), while Boma Mangbetu is down to 25%. Tshopo is following up with 94 out of 115 contacts (81.7%), and South Kivu has seen all 55 of its contacts, including 44 in cantonment.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 7 alerts notified

Seven alerts have been issued to crossing points in the province in the last 24 hours.

North Kivu — 6 alerts notified

Six alerts were notified to the province's activated PoE/PoC points.

Tshopo — not reported

No alerts were notified to the province's activated PoE/PoC devices.

Haut-Uélé — 0 alerts notified

No alerts were notified to the province's activated PoE/PoC devices.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri North Kivu		South Kivu	Tshopo	High-Uele	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (18/18)	100% (9/9)	21.1% (4/19)	30.3% (10/33)	72.7%
Number of people who have switched to PoE/PoC	108,393	ND	0	1,178	0	109,571
% of travelers screened % of	96.5%	98.0%	0	99.9%	0	96.5%
travelers who washed their hands	96.3%	ND	0	99.4%	0	96.3%
% of travelers made aware	99.2%	ND	0	100.0%	0	99.2%
Number of alerts notified	7	6	0	0	0	13
Number of validated live alerts	4	0	0	0	0	4
Lifeless bodies intercepted today	2	0	0	0	0	2
Problem contacts intercepted today	0	0	0	0	0	0
% of validated live alerts referred to CT/CTE	25% (1/4)	0% (0/6)	0	0	0	25% (1/4)
% of lifeless bodies swabbed	50% (1/2)	0	0	0	0	50% (1/2)
Positive results from referred suspected cases	0	0	0	0	0	0

4 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that have received interventions	10/28 (35.7%)	4/4 (100%)
Confirmed cases are being monitored.	128/181 (70.7%)	7/7 (100%)
New suspected cases	31 / 47 (66.0%)	4/4 (100%)
Suspected cases are being monitored	97 / 109 (89.0%)	52 / 23 (100%)
Laboratory results announced (including positive ones)	(12)	11 (4)
Children separated — nursery / community	1/52	0 / 0
CTE support staff	301	5
Frontline Personnel (PPL)	30	0
Members of households and communities	86	0
EDS supported (SMSPS)	17	ND

5 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD IN ITURI

PPL = healthcare personnel and frontline providers.

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Nia-Nia	22	5	12	5
Mongbwalu	17	7	1	9
Nyankunde	14	11	1	2
Lita	4	1	2	1
Mangala	4	2	1	1
Nizi	3	0	0	3
Logo	3	0	3	0
Bamboo	2	2	0	0
Kilo	2	1	1	0
Damascus	1	0	0	1
Total — case fatality rate 30.7%	140	71	23	43

6 Pillar-Based Response Activities

COORDINATION

• Organization of the workshop for updating the response plan in the context of the 5 affected provinces.

• **North Kivu:** production of the provincial SitRep n°73; participation in the coordination meeting of the Musienene ZS and evaluation of the functionality of the zonal coordination; tripartite meeting BCZ – MSF France – DPS delegation on the rehabilitation of an isolation center at the Musienene HGR; four recommendations retained, including the dissemination of accreditation notes and the commitment of morgue managers before August 6.

• **Haut-Uélé:** Morning coordination meetings held (2/2, 100%) and zonal reporting completed by 4 out of 5 health zones (80%); of the nine recommendations followed, three have been implemented — transmission of the lists of members of the

CONTINUITY OF ESSENTIAL SERVICES (CSSE)

• **Ituri:** 28 ambulances of which 24 are available, 46 trips made and 45 successful for 45 patients evacuated; 3 ambulances remain out of service (Bambu 1, call center 2 and Rwampara 1) and the other health zones have not reported their movements.

• **North Kivu:** risk mitigation ensured on five interventions (decontamination of CS Saint-Luc, PCI activities at the Saint-Vincent clinic and the Dalson medical center, burial at the Masiani cemetery, security assessment of the AS of Mabolio and Cité Belge), all covered at 100%; four health areas of Beni are stable and nine remain classified as problem areas.

COUSP, weekly analysis of alerts and deaths, securing the new laboratory and CTEs by the PNC —, one is underway and five remain undone. ŷ Tshopo: The three recommendations of August 1st have been implemented; three new recommendations for immediate implementation concern the tracing of an Ebola death that occurred on a boat departing from Kisangani between July 19th and 21st, the persistent resistance of high-risk contacts of the deceased body brought to Kabondo, and the impending shortage of PPE and laboratory supplies. The Avakubi PoC has been activated and Bafwasende has been established.	
LOGISTICS ŷ Ituri: Delivery of PECH kits to the Nia-Nia Health Zone and office supplies to the PECH pillar; currently being delivered, PECH kits and construction materials to the Lita Ebola Treatment Center (ETC) and WASH (Welthungerhilfe) kits to Komanda and Nyankunde; IPC/WASH and EDS inputs being prepared for Mongbwalu. 49 vehicles and 10 ambulances are allocated to the pillars, including 15 for IPC/EDS and 8 for surveillance. Work on the Rwankole ETC is complete, although its launch has been postponed to August 4, and 7 tents have been installed at the Nia-Nia ETC. ŷ North Kivu: an ambulance was stoned in the Beni health zone while picking up a suspect at CH La Joie (AS Kanzulinzuli), without major damage thanks to the intervention of the FARDC and the PNC; construction work continues at the Musienene (MSF France) and Beni (MSF Holland) treatment centers; the visit to the Oicha treatment center reveals a capacity of 10 beds occupied at 100%, insufficient PPE and an unprotected site. ŷ Tshopo: four liaison vehicles mobilized, including three provided by the WHO, and two vehicles allocated to the teams of CDC Africa and CDC Atlanta; start of work on the large capacity CTE (delimitation of the area to be cleared and recruitment of workers), slowed by a blockage by the site manager; water supply at PK 23.	PCI / WASH ŷ Ituri : 24 rings opened, 27 ESS monitored and supported (100%), 7 ESS and 9 means of transport decontaminated, and 49 households of cases decontaminated within 48 hours out of the expected 75 (65%). 112 PPL trained (Nia-Nia 100, Lita 12) and 84 RECO and community leaders briefed. The cumulative number of infected PPL remains at 134 out of 2,884 exposed (4.6%), with no new cases today. ŷ North Kivu: 3 households with confirmed cases decontaminated out of 47 (Katwa 1, Butembo 2) and 14 health establishments decontaminated; 10 health establishments with incomplete infection prevention and control (IPC) kits, 189 health establishments monitored and 807 service providers briefed; 3 health establishments assessed using scorecards — 79.9% in Musienene versus 27.4% in Beni; 79 risk assessments of staff exposure, including 58 high-risk areas. Training of 20 supervisors and 20 focal points in Kalunguta and 50 supervisors in Katwa, Musienene, Butembo, and Vuhovi completed with WHO support. ŷ Tshopo: IPC assessment of the Boyoma Health Center (score 17.7%) and decontamination of the Maman Régine dispensary (Lubunga) which received the suspected case, with incineration of mattresses and supplies used; integrated training of 85 riverboat operators (WHO "River without Ebola" project) and service providers from Bengamisa. The assessment of the Kongakonga displacement site (5,853 people, 1,342 households) revealed 16 latrines, 11 showers, no handwashing facilities in the 46 tents, and 39 torn tents. Incinerator at Makiso-Kisangani General Referral Hospital in breakdown.
COMMUNICATION (RCCE / CREC) ŷ Ituri: 5,060 people reached at home during 17 visits, 22 during 7 educational talks, 30 through mass awareness campaigns, and 32 via 3 advocacy meetings; 119 community alerts reported (109 living, 10 deceased) in Nyankunde, Rwampara, and Tchomia. Out of 103	HOLISTIC CARE (HCC) ŷ Ituri: 21 new recoveries; 471 patients in isolation (302 confirmed, 169 suspected) out of 833 beds, representing 56.5% occupancy, and 57 severe or critical cases, with 77 admissions and 75 discharges. Two treatment centers are exceeding capacity for confirmed cases — Nizi (272%) and ISTM

Of the feedback collected, 37 were clarified (36%). A refusal persists in Rwampara and reluctance in Nizi, where a 60-year-old woman who lost 10 family members refuses her transfer; the resistance in Nyankunde, linked to the delay of the EDS teams, has not been overcome. ŷ North Kivu: 19,379 households visited out of 25,500 (75.9%) and 32,605 people reached through door-to-door visits, 3,199 during dialogues and discussions, and 3,803 in public places; 62 out of 72 radio stations broadcast (86.1%) and 165 alerts of suspected individuals were received out of 450 expected (36.6%); 6 out of 8 feedback issues were resolved within 48 hours and the 2 reported incidents were addressed. Mass communication was conducted in Butsili (Beni, 81 participants), community dialogue in Mbimbi (Oicha, 42 breastfeeding women), and an interactive program was broadcast on Kantine radio (Biena). ŷ Tshopo: Awareness-raising at the Kisangani central market by URNSC youth (1,100 participants), support for IT training in the city's five health zones (92 participants) and for stakeholders in the river police and shipowners (85 participants); 17 community alerts collected and continued communication around the cases of Kongakonga and Kabondo. ŷ South Kivu: the community-based door-to-door communication strategy totaled 44,585 households visited out of 57,638 (77%), 121 patients referred to health centers and 4 visitors from identified affected areas; Nyangezi reached 100% coverage on its six health areas, Ibanda 68%. ŷ Haut-Uélé: 1,271 people reached at home in 458 households and 5,500 in mass awareness campaigns, including support from the governor in Vube (Rungu) and in the Mayogo-Magbai chiefdom; 4 out of 14 radio stations broadcast and 50 IEC tools were distributed, bringing the total to 285. Educational discussion with the 11 family members of the deceased at the Eastern Clinic, awareness-raising of 38 members of the ECZ in Pawa and discussions around 2 recovered patients in Wamba; 7 feedback sessions recorded, 5 of which were recorded.	(140%) — and one for suspected cases (Aru 100%). Nutritional support: 3 patients on a specific diet and 1 under management of severe acute malnutrition. ŷ North Kivu: 4 confirmed cases and 18 suspected cases were admitted, resulting in 2 deaths and 9 discharges without confirmed cases; no recoveries. At the end of the day, 196 patients were in isolation (27 confirmed, 169 suspected) out of 141 beds. An assessment mission was conducted at the Katwa Ebola Treatment Center (ETC), Matanda Hospital, and the Guadeloupe University Hospital Center (CUG). Regarding nutrition, 27 nutritionists were recruited for 9 health zones (IEDA Relief/UNICEF), and joint supervision was carried out by APATEMACO/WFP in Butembo, Katwa, Beni, and Oicha. Masereka and Mabalako remain without nutritional support. ŷ Tshopo: One new admission—a 13-year-old suspected case at the Kisangani Hospital Treatment Center—brings to 3 patients in isolation at the end of the day (1 confirmed, 2 suspected) out of 20 beds (15.0%); continuation of experimental treatments for 2 high-risk contacts of PPL in Madula, one of whom has been discharged, and for professional contacts at Kisangani Hospital, one of whom has had treatment discontinued. Eight high-risk contacts in Kongakonga refuse treatment. Nine hot meals served at the Treatment Center. ŷ Haut-Uélé: 31 patients in isolation at the end of the day out of 68 beds (45.6%); the daily patient flow chart shows no patient admissions, so admissions and discharges are not documented. Four suspected cases were transferred to the Isiro CT/CTE and three suspected cases from AS Babonde were evacuated to the Pawa General Referral Hospital, where the establishment of a transit center is considered urgent.
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Deployment of field investigators with support from Africa CDC and preparation of qualitative research in the health zones of Bunia and Rwampara; 11 protocols validated by the ethics committee (100%), 3 studies conducted out of 11 planned (27.2%), 3 reports produced and 1 publication; 4 recommendations implemented out of 22 (22.7%). No vaccination data are reported.	Dignified and secure burials (DSM) ŷ Ituri: 78 death alerts, 69 EDS (Emergency Depletion Scores) carried out, including 13 in Ebola Treatment Centers (ETCs), and 55 bodies recovered; 13 out of 32 zones reported (40.6%). Resistance persists in Bambu (family refusal) and Mambasa (disbelief in Ebola virus disease). The cumulative total has reached 2,779 EDS for 3,424 alerts since May 20. The formation of two additional teams has begun in Lita.

	ŷ North Kivu: 41 death alerts, 41 bodies swabs and secured, 42 DHSs carried out and no failures; 15 bodies remain pending. Deaths occurring in SSE areas predominate, in Beni (14) and Katwa (12). The IPC pillar reports 80 swabs and 52 DHSs on the same day — a discrepancy that needs to be reconciled. ŷ Haut-Uélé: 5 death alerts received (Wamba 2, Isiro 2, Pawa 1), 5 bodies swabés and 5 EDS carried out, no failures or postponements; acceptance by the two bereaved families of Isiro. Chlorine stock shortage and delay in digging the pits. ŷ South Kivu: no community deaths or post-mortem swab reported during the day; 16 suspects are being treated at CT (Katana 7, Kadutu 5, Miti-Murhesa 4) and 44 contacts remain in cantonment.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ŷ Ituri: Community-based briefings and awareness-raising sessions on PSEAH/GBV have begun in the Bambu Health Zone thanks to trained focal points; briefings for stakeholders and signing of the code of conduct by 20 participants (2 women, 18 men) from the ECZ and pillars of the Fataki Health Zone. Challenges: incomplete coverage of affected areas and lack of complaint mechanisms (CBCM/CFM) in several of them.	

7 Key challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient contact tracing performance: Ituri 83.4%, North Kivu 75.0% and South Kivu 100%, the Haut-Uélé and Tshopo did not provide data; the health zones of Damas, Logo and Kambala (Ituri) remained without usable data	Uninterrupted transmission chains Train, deploy and motivate community liaisons (RECOs); strengthen supervision; consolidate the recovery observed in Ituri
4	Ambulance shortages and pre-positioning at points of control	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS

Challenge No.		Impact	Actions required
5	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (181/1170, i.e. 15%)	Risk of nosocomial infections (140 cases of PPL, including 41 deaths in Ituri; 11 PPL, including 3 deaths in North Kivu and 3 PPL confirmed at Haut-Uélé)	Urgent supply, strengthening of IPC training
6	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
7	Installation of local laboratories (Rwampara, Fataki, Bambu-mine, Boga and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
8	Inadequacy of the resources financial	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
9	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
10	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities

Awareness-raising of 38 members of the ECZ of PAWA (including 8 women).



Awareness-raising at the Bethesda Centre/People living with disabilities in AS N'sele



FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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